MECHANICALLY VENTILATED / CRITICALLY ILL PATIENT - DETAILED NOTE

Patient received in bed, mechanically ventilated via ETT size ____ fixed at ____ cm. Conscious level/GCS ____/15; sedation score _____ if applicable. Head-to-toe assessment completed. Ventilator mode _____, FiO2 ____%, PEEP ____, RR ____, TV/PS _____ as prescribed. SpO2 ____%. Bilateral air entry _____; suction performed as clinically indicated with response _____. ETT position/fixation checked and secure. Ventilator circuit/connections checked, patent, supported and free from kinking/tension. HOB maintained at 30-45 degrees unless contraindicated. Oral/airway care provided as indicated.

Cardiac rhythm _____, HR ____, BP ____/____, MAP ____. Peripheral perfusion/pulses _____. IV/arterial/central lines assessed: _____. Dressings clean/dry/intact [or document findings]. Infusions _____ running as prescribed. Drains/CBD/NGT/other devices _____ checked, secured and patent as applicable. Intake/output documented.

Repositioning Documentation

Patient assessed prior to repositioning. Current position and skin/pressure areas checked. Patient repositioned to right 30° lateral / left 30° lateral / supine position according to the individualized repositioning schedule and clinical condition. Head of bed maintained at ____° as clinically appropriate. Proper body alignment maintained. Appropriate lifting/turning technique used to minimize friction and shear; patient not dragged across the bed. Pillows/positioning aids placed appropriately to support the head/neck, back, upper and lower limbs and to prevent direct pressure between bony prominences. Heels completely offloaded from the mattress when indicated.

Skin assessed during repositioning, including occiput, ears, shoulders/scapulae, elbows, sacrum/coccyx, greater trochanters, ischial areas, knees, ankles/malleoli and heels as applicable. Skin condition: _____. No new non-blanchable redness, discoloration, moisture-associated skin damage or skin breakdown noted / abnormal finding: _____.

Suctioning: Oral / endotracheal / tracheostomy suctioning performed as indicated. Secretions noted: color _____, amount (scant / small / moderate / large) _____, consistency (thin / thick) _____. Patient tolerated the procedure well. SpO_2 maintained at ____% / changes noted: _____

Positioning — Comprehensive Bedside Entry

At ____ hrs patient assessed before turn; initial position _____, comfort/pain _____, SpO_2 ____%, HR ____, BP ____/____. Repositioned to _____ (supine/right 30° lateral/left 30° lateral/other clinically appropriate position). Head of bed elevated ____° as clinically appropriate. Head/neck/spine and limbs aligned; pillows/positioning supports placed at _____ to protect bony prominences and avoid pressure between knees/ankles. Heels fully offloaded when indicated using _____. Lift/slide technique used to reduce friction and shear; patient not dragged. Skin assessed at accessible pressure areas: occiput/ears _____, scapulae/elbows _____, sacrum/coccyx _____, hips/trochanters _____, knees/malleoli _____, heels _____. Moisture/redness/non- blanchable discoloration/skin breakdown: _____. Skin

under/around medical devices checked: _____. IV/CVC/arterial lines, urinary catheter, drains and oxygen/ETT/ventilator circuit secured and checked before/after turning as applicable: _____. After turn SpO_2 ____%, HR ____, BP ____/____; breathing/comfort _____. Patient tolerated _____.

PRESSURE INJURY PATIENT

Skin assessment completed. Existing pressure injury located at _____ (exact anatomical/bony prominence), Stage _____. Wound size ____ x ____ x ____ cm [if measured], wound bed _____, exudate _____, surrounding skin _____. Braden score ____; reassessed this shift. Direct pressure over affected area avoided. Patient repositioned to _____ according to individualized care plan. Appropriate offloading/pressure- relieving device _____ used; heels offloaded as indicated. Skin kept clean and dry; moisture/friction/shear prevention measures maintained. Dressing/wound care performed as ordered using _____; patient tolerated _____. PUSH assessment score ____ / updated as required. Patient/family education provided regarding repositioning, skin care and pressure injury prevention as applicable. New/deteriorating findings _____; Dr./in-charge/wound team informed at ____; advice/order _____. Reassessment/response _____. Pressure injury status included in SBAR/handover.

Positioning For Bedridden With Limited Mobility:

Patient is partially bedridden with limited mobility, requiring assistance with daily activities. Able to change position in bed with minimal assistance. Repositioning encouraged and assisted as needed. Skin and pressure areas assessed(write the assessment). Fall prevention measures maintained.

PATIENT REFUSED REPOSITIONING

Repositioning offered at ____ and importance of regular position change/pressure injury prevention explained. Patient declined repositioning due to _____. Risks explained and patient verbalized understanding [if applicable]. Skin/pressure areas assessed as tolerated: _____. Comfort measures provided. Will continue to encourage/reassess. In-charge/physician informed if clinically indicated: _____.

Desaturation / Instability During Repositioning

Repositioning initiated at ____ with airway/lines/devices secured. During turning, SpO2 decreased from ____% to ____% / BP changed from _____ to _____ / patient developed _____. Repositioning stopped/position adjusted and patient reassessed. ETT/airway, ventilator circuit and connections checked as applicable. Intervention provided: _____. Dr./in-charge/RT informed at ____; advice/order _____. Following intervention, SpO2 ____%, BP _____, patient condition _____. Continued close monitoring.

Cardiac-Monitored Patient — No Pain

Patient received at ____ hrs, awake/alert _____, oriented. Connected to continuous cardiac monitoring; rhythm _____, heart rate ____ bpm, BP ____/____ mmHg, RR ____/min, SpO_2 ____% on _____, temperature ____°C. Patient denies chest pain, palpitations, dizziness, dyspnea or other symptoms at time of assessment / reports _____. Pain score ____/10. IV access/site _____. (fall)

Cardiac-Monitored Patient — With Pain

At ____ hrs patient reported pain at _____, onset _____, duration _____, score ____/10, character _____, radiation _____, aggravating/relieving factors _____. Associated symptoms (e.g., dyspnea, diaphoresis, nausea, dizziness): _____. Vital signs BP ____/____, HR ____, RR ____, SpO_2 ____%, rhythm _____. Patient assessed promptly; 12-lead ECG performed if indicated/ordered at ____ hrs, findings communicated _____. physician notified at ____ hrs; orders/advice _____. Prescribed intervention/medication _____ administered at ____ hrs with monitoring for response/adverse effects. Repeat assessment at ____ hrs: pain ____/10, BP ____/____, HR ____, SpO_2 ____%, rhythm _____; associated symptoms _____. Further escalation/intervention _____. Patient education _____; close monitoring continued and handover updated.

Lines — Peripheral IV

Peripheral IV cannula at _____. Site assessed for pain, redness, warmth, swelling, leakage, infiltration, phlebitis and dressing integrity: _____. Cannula secured and patent as assessed; infusion/medication _____ at ordered rate _____; line/tubing and connections checked _____.

Lines — Central Venous Catheter

CVC type/site _____, lumens ____. Site/dressing assessed: clean/dry/intact or finding _____; erythema, tenderness, discharge, bleeding or swelling _____. line secured, clamps/caps/connections checked, lumen patency assessed as appropriate _____. Infusions _____ via lumen _____, rates _____.

Drain Care:

Drain assessed and maintained in place as ordered. Insertion site checked for redness, swelling, leakage or signs of infection. Dressing clean, dry and intact / _____. Drain secured, patent and functioning properly. Drainage noted: amount ____ mL, color _____, consistency _____. No kinking, blockage or dislodgement noted. Drainage output monitored and documented in the intake and output chart. Patient tolerated drain care well.

Bed Bath

Complete bed bath provided with full assistance, including hair shampooing, oral hygiene, perineal care, back care, foot care , eye care and skin moisturizing. Patient's level of consciousness, cooperation, and ability to participate in hygiene care assessed. Privacy and dignity maintained throughout the procedure. CBD care performed using aseptic technique, and catheter insertion site assessed for redness, swelling, discharge, or other signs of infection. Full skin assessment performed, including inspection of bony prominences (sacrum, coccyx, heels, elbows, hips, and greater trochanters) for redness, pressure injuries, or skin breakdown. Skin condition documented(--------), and Braden Scale reassessed according to policy. Clean bed linens and patient gown changed. Patient repositioned using appropriate moving and handling techniques, with pressure areas offloaded and pillows provided for support. Head of bed elevated as clinically appropriate.

CCU Admission Note for Ventilator patient

Patient: Mr. X Received From: Accident & Emergency (AE) Mode of Transfer: Stretcher, accompanied by Doctor, Staff Nurse, Medical Service Aid, and Respiratory Therapist with portable ventilator &Cardiac monitor.

Ventilator Settings:

* Mode ….

* FiO_2: …

* Tidal Volume (TV): …

* Respiratory Rate (RR): …

* PEEP: …

* Pressure Support (PS): …

* SpO_2: …

Neurological Status:

* Patient conscious but sedated / unconscious

* GCS score …..

Transfer of Pt to CCU bed

Pt safely transferred to CCU bed .Unit orientation not given as patient is fully dependent and sedated/unconscious

Monitoring & Devices:

* Cardiac monitor connected ….. rhythm

* HR: …, BP: …, Temp: …peripheral pulse absent/present

* Endotracheal tube size ….. connected to bedside ventilator; settings reassessed by respiratory therapist. No respiratory distress oserved.

* ET tube lip level: … cm

* Air entry bilaterally equal and good

* NGT {size …} and CBD [size…}in situ

* All lines secured as per hospital guidelines

Infusions and site observation

IV infusions…….. via right/left femoral/subclavian/jugular/peripheral line .No bleeding, infiltration, or phlebitis noted.

Investigations & Assessments:

* ECG done – shows …

* Morse Fall Assessment: Score … (….. risk) → Fall prevention guidelines implemented

* MUST Score: …[….Risk}

* AEWS Score: …{…Risk}

* Comprehensive skin and head-to-toe assessment done {e.g. noted lower limb edema, abrasions, rashes, hematoma, etc.}

* Pressure injury noted in ……region (Length …, Width …exudate.., Stage …, PUSH tool score …). Air mattress applied.

* Patient kept in ….. position with head-end elevation at 45°(30° for non-ventilated pts){Lt lateral, Supine, Rt lateral}Bony prominences protected and heels off loaded. All attached devices checked and secured.

* Braden Assessment: Score … (… risk)

Notifications:

* Senior on-call doctor notified of admission

Advice: Continue same treatment or new orders if any.

Additional documentation

* Admission weight not checked as patient is bedbound

* Identification band kept on side rails as both hands are edematous {if indicated}

* Present history, past history, social history &Family history

* Health education aspect to the family

Additional documentation for acute stroke pts

* Level of consciousness

* GCS
* Oriented/Disoriented
* Speech or language
* Facial symmetry
* Pupils and reaction
* Limb strength
* Sensation

Additional documentation for Psychiatric pts

* Level of consciousness
* Oriented/Disoriented
* Behaviour
* Speech
* Hallucinated or not
* Cooperation with care